Clinical Care-Plan Case Study: Diabetic Foot Infection with Osteomyelitis, Before and After First-Ray Amputation

ADN Med-Surg II | Individual take-home clinical care-plan case study | Open book

Your patient

You are the primary nurse for the patient in Medical-Surgical 4 East, room 436-1, on the 7:00 to 15:00 shift on Wednesday, October 21, 2026 (hospital day 3). Open the patient in the student EHR (Student View, or the patient file from your instructor) and set the simulation clock to 0700. Everything you need is in the chart. He is waiting to go to the operating room for surgery on an infected foot, and he has several other chronic illnesses. Work the way you would at the start of a real shift: before the surgery, while he is in the OR, and when he comes back.

How this case study works

This packet follows the nursing process: Assessment, Diagnosis, Planning, Implementation and Evaluation. You will also explain the pathophysiology and pharmacology behind your patient's care. Expect to spend about 5 hours. It is a take-home, individual, open-book assignment: use your textbooks, drug guide, notes and the chart.

Questions ask you to explain WHY, not only WHAT. Short, specific answers that use this patient's data earn more points than general textbook answers. Real charts are imperfect: you will be asked to catch conflicts and to question orders and recommendations before you act.

Part	Nursing process step	Estimated time
1	Assessment: Patient Data and Chart Review (Assessment)	60 min
2	Pathophysiology: The Disease Process (Assessment / Analysis of cues)	60 min
3	Pharmacology and Medication Safety (Assessment / Analysis (and safe implementation))	50 min
4	Nursing Diagnoses and Priorities (Diagnosis)	25 min
5	Planning and Implementation (Planning / Implementation)	50 min
6	Evaluation: What Happened and What Next? (Evaluation)	25 min
7	Clinical Judgment (Next-Generation NCLEX Style) (Evaluation / Clinical judgment)	30 min
	Estimated total	**5 h 0 min**

Scoring

- The packet is worth 100 points. A score of 78 or higher is passing.
- Each numbered item shows its point value. Points are given only for what is written in the box.

Writing so your pages can be scored from photos

- Write in dark pen or very dark pencil, print clearly, and stay inside the box. Write the answer in the box with the same number as the question.
- Write your name and date at the top of every page (the header prints a line for it).
- If you need more room, write "continued on the back" and put the item number on the back. Do not squeeze writing between boxes.
- Do not use abbreviations that are not on the approved list. Spell out drug names.
- When you finish: photograph EVERY page, one page per photo, on a flat surface in good light with all four corners and the page number in view. Check that every photo is readable before you upload.

Part 6 is sealed

- Do not read the sealed page at the start of Part 6 until Parts 1 to 5 are finished. Your plan must be based on what you knew at 0700.

Honor statement: I completed this work myself, using only my textbooks, notes, drug references and the patient chart.

Signature: _____ Date: _____

PART 1: ASSESSMENT: PATIENT DATA AND CHART REVIEW

Nursing process step: Assessment | About 60 minutes | 26 points

Collect and organize the data you would have at 0700. Where the chart does not document something, write "not documented." Use R and L for laterality.

1.1 Basic Patient Information *(3 pts)*

Where to look in the EHR: Patient Summary and Storyboard

Room Number:		**Age:**	
Gender:		**Admission Date:**	
Allergies (and reactions):		**Chief Complaint:**	
Primary Diagnosis:		**Code Status:**	
Isolation:		**Height / Weight:**	

1.2 Medical Diagnoses and How They Affect the Foot Infection and Healing *(3 pts)*

List FOUR other medical diagnoses from this patient's chart. For each, explain how it affects his foot infection, his surgery or his healing.

Where to look in the EHR: Chart Review > H&P; Encounters > Hospital problem list

Medical diagnosis	How it affects the foot infection, the surgery or healing

1.3 Patient History *(3 pts)*

Where to look in the EHR: H&P; ED Provider Note; Case Management Initial Assessment; Nursing Admission Note

Medical History:

Write one answer per numbered box, inside the box, in dark pen, neatly. Open book, individual work.

Surgical History:

Family History:

Social History:

Home Situation and Discharge Needs:

Pre-Hospital Story of Patient:

1.4 Head-to-Toe Assessment at 0700 (use R and L for laterality) *(4 pts)*

Document the findings from the chart as of the start of your shift. If the chart does not document a finding, write "not documented."

Where to look in the EHR: Flowsheets; Nursing Admission Note; Nursing Progress Note 0645

Neurological (include foot sensation):

Cardiovascular and perfusion: Radial pulses R___ L___ Pedal pulses R___ L___ Cap refill R foot ____

Respiratory: breath sounds, effort, O2

Integumentary (R foot wound: size, tissue, drainage, erythema; other skin):

Musculoskeletal, mobility and pain:

Gastrointestinal and Genitourinary (diet/NPO, I&O):

IV / Lines / Devices Assessment:

Psychosocial / Cultural Assessment and Erikson Stage:

1.5 Vital Sign Trend *(2 pts)*

Copy the vital signs at each time. Circle every abnormal value. Then describe the trend in one sentence.

Where to look in the EHR: Flowsheets > Vital Signs (or the Vitals graph)

Vital sign	10/19 0206	10/20 1200	10/21 0400

Write one answer per numbered box, inside the box, in dark pen, neatly. Open book, individual work.

	(ED arrival)		
Temperature			
Heart rate			
Blood pressure			
Respiratory rate			
SpO2 (%)			
Pain (0-10)			

Trend in one sentence:

1.6 Laboratory Data *(3 pts)*

Choose the 6 laboratory results most important for this patient today. For each give the most recent value, the normal range, the trend since 10/19, and explain why it matters for THIS patient.

Where to look in the EHR: Lab Results (look at the trend, not only today)

Lab	Latest value, normal range, trend	Why is this lab important for this patient?

1.7 Diagnostics *(3 pts)*

Where to look in the EHR: Chart Review > Imaging, Cardiology, Notes (arterial studies), Labs (microbiology)

X-ray right foot: results

MRI right foot: results (note whether contrast was used)

Arterial studies (ABI, toe pressure): results and what they mean

ECG / echocardiogram / chest x-ray:

Cultures and MRSA screen:

Urinalysis:

1.8 Chart Reconciliation *(5 pts)*

Real charts contain conflicts, copy-forward errors and gaps. Find FOUR places where this chart is conflicting, unsafe, or needs clarification before you act. Say where you found each and who you would clarify with.

Where to look in the EHR: Orders, MAR, Chart Review > Notes (ID and hospitalist notes), Devices, Sticky notes

What is wrong or unclear (and where)	Who you would clarify with and what you would ask

PART 2: PATHOPHYSIOLOGY: THE DISEASE PROCESS

Nursing process step: Assessment / Analysis of cues | About 60 minutes | 20 points

Explain WHY this is happening. Use your own words (plain language) and tie each idea to this patient.

2.1 Pathophysiology of a Diabetic Foot Infection with Osteomyelitis *(5 pts)*

In plain language and your own words, explain the pathophysiology of this patient's primary diagnosis: why a small ulcer under his foot went unnoticed, why it did not heal, how it reached the bone, what the body does in response, and why the problem is treated with surgery as well as antibiotics.

2.2 Cause-and-Effect Chain *(3 pts)*

Fill in each box (one or two lines) to show how events in this patient lead from long-standing diabetes to the problems you will have to prevent.

1. Why can he not feel his foot? (factors in THIS patient)	
▼	
2. Why did the ulcer start and stay open for 3 weeks?	
▼	
3. Why is healing poor? (name at least 3 patient factors)	
▼	

4. How did the infection reach the bone?	

▼

5. What is the body's response (data from his chart)?	

▼

6. How do surgery, antibiotics and offloading interrupt the chain?	

2.3 How Do You Know? Findings and Mechanisms *(3 pts)*

List SIX findings from this patient's assessment, labs or diagnostics that support the diagnosis or show how his body is responding. Beside each, explain the mechanism that causes it.

Where to look in the EHR: Notes, Flowsheets, Lab Results, Imaging

Finding in THIS patient (with value)	Why it happens (mechanism)

2.4a Perfusion: PAD and Healing *(2 pts)*

His right ABI is 0.72, but the x-ray shows dense calcification of the foot arteries, and the toe pressure is 58 mmHg. Explain (a) what the ABI and toe pressure tell you, (b) why the ABI can be misleading here, (c) why that toe pressure matters for surgery, and (d) why SCDs/compression stockings are not allowed and how smoking harms him.

2.4b Kidneys: CKD Stage 3 *(2 pts)*

His creatinine is 1.98 and eGFR 38 (baseline creatinine about 1.8). Explain FOUR ways his kidney disease changes the care in this chart (include drugs, other nephrotoxins, and IV access for 6 weeks of antibiotics).

2.5 Treatment *(2 pts)*

What are we doing to treat this diagnosis before, during and after surgery? Include medical treatments AND nursing monitoring.

2.6 Reading the Lab Trends *(3 pts)*

Look at WBC (17.8, 14.7, 13.6), creatinine (1.94, 1.91, 1.93, 1.98), glucose (336, 214, 185), vancomycin level (18.9, 18.9), hemoglobin (10.8, 10.5, 10.0), albumin (3.3, 3.2, 3.1) and CRP/ESR (measured once). What is getting better, what worries you, what is missing, and what would you tell the surgeon and pharmacist before the 0900 antibiotics?

Where to look in the EHR: Lab Results (trend view)

PART 3: PHARMACOLOGY AND MEDICATION SAFETY

Nursing process step: Assessment / Analysis (and safe implementation) | About 50 minutes | 14 points

Use your drug guide and the MAR (and the drug guide link inside the EHR). Part of your job is to question orders and recommendations before you act.

3.1 Medication List *(4.5 pts)*

From the MAR and orders, choose the SIX medications that matter most to you before and after his surgery today. List brand and generic names.

Where to look in the EHR: MAR (click the medication name for the drug guide); Orders

Medication (brand and generic)	Drug class and mechanism (10 words or fewer)	Why is THIS patient taking it?	Key nursing check / hold parameter

3.2a Which Antibiotic? Penicillin Allergy and Stewardship *(2.5 pts)*

The ID consult and hospitalist notes recommend vancomycin PLUS piperacillin-tazobactam, but the MAR has vancomycin PLUS meropenem. (a) What do you do about the pip-tazo recommendation and why? (b) Why is meropenem acceptable, and what do you watch for? (c) What will decide whether he needs 2-5 more days or 6 weeks of IV antibiotics, and what cultures are still pending? (d) What do you raise about IV access for a 6-week course?

Where to look in the EHR: Allergies; MAR; Notes (ID consult 10/19 1300, H&P, hospitalist 10/20); Orders (ertapenem)

3.2b Vancomycin, the OR and the Kidneys *(2 pts)*

Vancomycin 1,000 mg is due at 0900, when he will be in the OR or PACU. (a) Read the hold parameters on the order and decide whether any is met today (show the values). (b) What is odd about how the levels were timed? (c) Whom do you call, and what do you do so the dose is neither missed nor given wrongly? (d) A colleague suggests ketorolac for post-op pain: what is your response?

Where to look in the EHR: MAR; Orders (vancomycin, renal dosing review); Labs

3.2c Insulin While NPO *(2 pts)*

NPO since midnight. Glucose was 185 on this morning's lab. Glargine 22 units is ordered at bedtime and lispro correction scale every 6 hours; the NPO order and the hospitalist plan say basal insulin 80% and mealtime insulin held. (a) What is the glucose goal? (b) Why is "hold all his insulin because he is NPO" wrong, and why is giving the full 22 units also wrong? (c) What do you check and when? (d) At 1130 he is sweaty and shaky and the meter reads 56 mg/dL: what do you do? (e) What changes in his insulin needs now that the infected bone is out, and what is unclear in the orders when he starts eating?

Where to look in the EHR: MAR; Orders (NPO, glargine, lispro, hypoglycemia protocol); Sticky notes

3.3 Math From the Chart *(3 pts)*

Show your work in the box. Include units.

Where to look in the EHR: MAR; Orders; Vital signs; I&O

a) The vancomycin loading dose was 2,750 mg and he weighs 108 kg. How many mg/kg is that? The rate must not exceed 1 g/hour: what is the shortest safe infusion time for the full load?

b) Pharmacy sends the 1,000 mg maintenance dose in 250 mL (this volume is given for the problem). To run it over 2 hours, what pump rate in mL/hour do you set?

c) His glargine order is 22 units. If he is still NPO tonight and the dose is reduced by 20%, how many units are due (round to the nearest whole unit)?

d) From 0300 to 0700 his urine output was 309 mL. What is that in mL/kg/hour for 108 kg, and is it above 0.5 mL/kg/hour?

PART 4: NURSING DIAGNOSES AND PRIORITIES

Nursing process step: Diagnosis | About 25 minutes | 8 points

Use NANDA-I diagnoses. Write each as problem, "related to" (cause you can treat), and "as evidenced by" (specific data). Do not use a medical diagnosis as the "related to."

4.1 Prioritized Nursing Diagnoses *(6 pts)*

Write FOUR nursing diagnoses in priority order (1 = highest) for the 0700-1500 shift.

#	NANDA-I nursing diagnosis	Related to (etiology)	As evidenced by (at least 2 specific chart findings)

4.2 Why This Order? *(2 pts)*

Explain why your #1 diagnosis is first (use ABCs, Maslow, or acuity). Name one lower-ranked diagnosis that could become the top priority and what finding would move it up.

PART 5: PLANNING AND IMPLEMENTATION

Nursing process step: Planning / Implementation | About 50 minutes | 14 points

Plan the care for the 0700-1500 shift. He goes to the OR at about 0800 and will return to the unit afterward. SMART goal = Specific, Measurable, Achievable, Relevant, Time-bound, written about the PATIENT (not the nurse).

5.1a Plan for Priority Diagnosis #1: _____ *(2 pts)*

I = independent, C = collaborative, D = delegated.

SMART goal for this shift	Nursing interventions (at least 4: what, how often)	Rationale	I / C / D

5.1b Plan for Priority Diagnosis #2: _____ *(2 pts)*

SMART goal for this shift	Nursing interventions (at least 4: what, how often)	Rationale	I / C / D

Write one answer per numbered box, inside the box, in dark pen, neatly. Open book, individual work.

5.2 Time-Sequenced Plan for the Shift *(3 pts)*

For each hour, list your priority actions IN ORDER. Include medications (and which you will question), pre-operative preparation, post-operative care when he returns, teaching and communication, and say why that time.

Where to look in the EHR: Brain/Worklist, MAR, Orders, Secure Chat, Notes

Time	Priority actions in order	Why at this time
0700		
0800		
0900		
1000		
1100		
1200		
1300		
1400		

5.3 Calling the Provider (SBAR) *(2 pts)*

At 0730 you call the hospitalist (Dr. Patel). The ID and hospitalist notes call for piperacillin-tazobactam although he is allergic to penicillin (the MAR has meropenem), vancomycin is due at 0900 during the OR time, the vancomycin levels were drawn at odd times (18.9 twice) and his creatinine is 1.98, and the glargine order is still 22 units while the plan says 80%. Write what you would say.

Where to look in the EHR: MAR; Orders; Notes; Labs

S: Situation

B: Background

A: Assessment

R: Recommendation / request

5.4 Delegation *(1.5 pts)*

Mark the LOWEST level of staff who may be assigned each task for this patient.

	RN only	LPN/LVN	UAP/PCT
Measure and record post-op vital signs and urine output, and float his heels on pillows as arranged by the RN	☐	☐	☐
Compare the pulses, color, warmth, sensation and movement of both feet and decide if the change is concerning	☐	☐	☐
Give the scheduled 1200 oral acetaminophen after the RN has checked that he is awake, swallowing and not nauseated	☐	☐	☐

5.5 Teaching and Discharge Plan *(3.5 pts)*

List FIVE teaching or discharge-planning topics this patient needs from admission to discharge. Adapt each to his barriers (lives alone, neuropathy, delivery driver, limited glucose checking, smoking) and say how you will check understanding.

Teaching / discharge topic	How you will teach it or arrange it (adapted to barriers)	How you will confirm learning

SEALED PAGE. Read only after Parts 1 to 5 are complete.

Part A: real chart notes written after 0700 (they are in the patient file; read them critically, they are not perfect). Part B (in blue) is the instructor addendum: it is NOT in the EHR; use it as if you had charted it.

10/21 08:30 Hospitalist Progress Note

Hospital Day 3
Interval events / subjective
Pain 3/10; understands the plan to remove the infected toe and bone. Worried about walking afterward.
- Right first ray amputation (great toe and first metatarsal head), open performed 10/21/26 0800: Purulent plantar abscess tracking along the first metatarsal head; infected, soft, discolored bone. Metatarsal cut back to firm bleeding bone; wound left open for infection control.

Objective
- Vitals (24 h): Tmax 99.5 F; HR 86-91; BP 153-160/85-88; RR 16-18; SpO2 96-97%; pain 2-3/10
- Current: T 99.3 F, HR 88, BP 158/85, RR 17, SpO2 96% on room air, pain 3/10
- I&O (24 h): in 2553 mL / out 2009 mL
- Diet: NPO; Activity: Bariatric / appropriate-size equipment and lift assist

Exam
- General: Awake and alert, in no acute distress.
- Cardiovascular: Sinus tachycardia; S1 S2 regular, no murmur. Edema: Right foot 1-2+ pitting edema.
- Respiratory: Clear bilaterally; unlabored, even.

10/21 09:54 Brief Operative Note

Pre-operative diagnosis
- Osteomyelitis of the first metatarsal head and great toe with plantar abscess

Post-operative diagnosis
- Osteomyelitis of the first metatarsal head and great toe with plantar abscess

Procedure
- Right first ray amputation (great toe and first metatarsal head), open

Surgeon / anesthesia
- R. Alvarez, MD; Popliteal and saphenous nerve block with monitored anesthesia care

Findings
- Purulent plantar abscess tracking along the first metatarsal head; infected, soft, discolored bone. Metatarsal cut back to firm bleeding bone; wound left open for infection control.

Description

10/21 10:42 PACU Nursing Note

PACU course
- Arrived in PACU at 0936 after right first ray amputation (great toe and first metatarsal head), open under popliteal and saphenous nerve block with monitored anesthesia care anesthesia.
- Vitals on arrival: T 99.3 F, HR 88, BP 158/85, RR 17, SpO2 96% on room air, pain 3/10.
- Airway patent, extubated, awakens to voice; Aldrete score 9 at discharge from PACU.
- Pain treated with IV opioid; nausea treated with ondansetron.
- Transferred to the surgical unit at 1042; report given to unit RN. Vitals at transfer: T 99.3 F, HR 88, BP 158/85, RR 17, SpO2 96% on room air, pain 3/10.

10/21 11:30 Physical Therapy Initial Evaluation

Prior level of function
- Lives alone; uses independent; works part-time as a delivery driver for mobility.

Current status
- Bed mobility and transfers: moderate assist of one.
- Gait: bed to chair with walker; steady with no loss of balance.

- Vitals with activity: T 99.3 F, HR 88, BP 158/85, RR 17, SpO2 96% on room air, pain 3/10; tolerated well.
- Pain: 3/10 with movement.

Assessment

- Decreased mobility and endurance related to acute illness; good rehab potential.

Plan / recommendation

- PT daily while inpatient; progress gait and stairs.
- Discharge recommendation: Home with home health PT/nursing, walker and offloading shoe (heel weight bearing right foot), or short-term SNF if unable to stay off the forefoot safely.

10/21 13:00 Occupational Therapy Initial Evaluation

Prior level of function

- Lives alone; needs assistance with some ADLs, uses independent; works part-time as a delivery driver.

Current status

- Grooming and upper-body dressing: set-up / supervision. Lower-body dressing and toileting: supervision.
- Cognition: Alert and oriented x4.

Plan

- OT 3-5x/week; energy conservation and ADL retraining; reassess equipment needs.

1115 INSTRUCTOR ADDENDUM (not in the EHR): back on 4 East from PACU

- The block is still working: R foot numb, toes warm, capillary refill of the 2nd toe 3 seconds, DP Doppler signal present. Dressing has a 2 cm spot of pink drainage on the outer wrap (you marked and timed it). Pain 2/10. Nauseated; ondansetron 4 mg IV given. Glucose 144 mg/dL.

1200 INSTRUCTOR ADDENDUM: diet advanced

- Glucose 118 before lunch (no lispro needed by the scale). He eats about 30% of the renal, consistent-carbohydrate lunch. Lisinopril, atorvastatin and senna-docusate start per the 1200 orders (BP 150/84).

1245-1315 INSTRUCTOR ADDENDUM: reassessment

- At 1245 he got up to the bathroom alone with the walker, without the post-op shoe, "because I can't feel my foot anyway." He did not fall. At 1315 the block is wearing off: throbbing pain 7/10 (oral/IV opioid PRN orders are available). The dressing now has about 6 cm of bright red strikethrough on the outer wrap (2 cm at 1115). The 2nd and 3rd toes look pale-dusky and cool, capillary refill 4 to 5 seconds, DP Doppler signal faint (it was clear at 1115). Enoxaparin 40 mg is next due at 2130.

Time	HR	BP	RR	SpO2	Temp	Pain	Other
0400 (chart)	88	158/85	17	96% RA	99.3 F	3/10	pre-op baseline; glucose lab 185 at 0500
1115	92	150/84	16	96% RA	99.1 F	2/10	glucose 144; 2 cm pink drainage; toes warm; cap refill 3 s
1315	106	168/94	20	95% RA	99.6 F	7/10	6 cm bright red strikethrough; toes 2-3 pale/cool, cap refill 4-5 s; DP Doppler faint
1330 labs							Hgb 9.2 (was 10.0); platelets 318; creatinine 2.01 (was 1.98); K 4.7; glucose 121

Next doses coming up after this shift: meropenem 2100 (order ends 10/22 0800), enoxaparin 2130, glargine 2100, ertapenem begins 10/22 0800.

PART 6: EVALUATION: WHAT HAPPENED AND WHAT NEXT?

Nursing process step: Evaluation | About 25 minutes | 8 points

Open the sealed page. It has two sections: REAL chart notes dated after 0700 (the EHR hides them until the simulation clock reaches them) and an INSTRUCTOR ADDENDUM that is NOT in the EHR. Do not change your Part 1-5 answers.

6.1 Evaluating Your Goals *(3 pts)*

For each goal you wrote in 5.1a and 5.1b, use data from the sealed page to say whether it was met, partly met, or not met, and what you will change.

Goal (copy from Part 5)	Data from the sealed page that you used	Met / Partly / Not	Revised plan

6.2 New Problems and Priority Actions *(3 pts)*

From the sealed page (both parts), name the TWO most urgent problems and for each the priority nursing actions (include who you notify, what you do about the 2130 enoxaparin and the pain medicine, and one thing you would question in the real post-op notes).

6.3 Focused Nursing Note *(2 pts)*

Write a focused nursing note for the 1315 event (DAR, SOAP, or your school's format): data, action, and response, with the plan.

PART 7: CLINICAL JUDGMENT (NEXT-GENERATION NCLEX STYLE)

Nursing process step: Evaluation / Clinical judgment | About 30 minutes | 10 points

Answer by marking the boxes. These items use the same patient. Some scenarios are hypothetical and say so.

7.1 Select All That Apply *(2 pts)*

Which nursing actions are correct for Mr. Jennings today, before and after his surgery?

Select ALL that apply.

- ☐ Float his heels on pillows and keep the right leg elevated without a pillow under the knee.
- ☐ Apply sequential compression devices to both legs to prevent clots.
- ☐ Inspect BOTH feet and between the toes every shift, not only the operated foot.
- ☐ Give ketorolac if he asks for something stronger than acetaminophen.
- ☐ Give the scheduled vancomycin and meropenem on time, arranging with the OR and PACU team how to do so.
- ☐ Hold all of his insulin because he is NPO.
- ☐ Use gown and gloves for dressing changes and room entry (MRSA contact precautions).
- ☐ Apply a heating pad to a cool right foot to improve circulation.
- ☐ Question any order for NSAIDs or IV contrast before giving or sending him.

7.2 Trend: Better, Same or Worse? *(2 pts)*

Using the lab results in the chart from 10/19 to 10/21, mark the trend for each row.

	Improving	No change	Worsening
WBC: 17.8, 14.7, 13.6	☐	☐	☐
Albumin: 3.3, 3.2, 3.1	☐	☐	☐
Creatinine: 1.94, 1.91, 1.98 (baseline about 1.8)	☐	☐	☐
Blood pressure: 152/83, 159/87, 158/85 (goal below 140/90)	☐	☐	☐

7.3 Bowtie *(2 pts)*

HYPOTHETICAL (not in the chart): fifteen minutes after his vancomycin 1,000 mg infusion is started over 30 minutes by a covering nurse, he has flushing and itching of the face, neck and upper chest. HR 108, BP 96/58, SpO2 97% on room air. No wheeze, stridor, lip or tongue swelling, or hives elsewhere.

Actions to Take (select 2)	Condition the patient is most likely experiencing (select 1)	Parameters to Monitor (select 2)

☐ Stop the infusion, stay with him and notify the provider ☐ Restart later over at least 2 hours once symptoms resolve and the provider agrees (antihistamine if ordered) ☐ Give epinephrine 0.3 mg IM immediately ☐ Increase the infusion rate to finish sooner ☐ Continue at the same rate and recheck in 30 minutes	☐ Vancomycin infusion reaction ☐ Anaphylaxis to penicillin ☐ Hypoglycemia ☐ Surgical site infection	☐ Blood pressure and heart rate ☐ Respiratory effort, wheeze, stridor and lip or tongue swelling ☐ Right foot capillary refill ☐ Bowel sounds ☐ Urine specific gravity

7.4 Recognize Cues *(2 pts)*

Excerpt from a HYPOTHETICAL post-operative day 1 shift report. Which statements need follow-up NOW?

Draw a line under each statement that needs follow-up now. Then write the item numbers in the box below.

(1) Glucose 156 before breakfast; no symptoms of hypoglycemia.

(2) The foot dressing has bright red drainage soaked through to the outer wrap and now extends past the line marked at 0400.

(3) Pain 4/10 at rest, 2/10 about 45 minutes after oxycodone.

(4) The remaining 2nd toe is pale and cool with capillary refill of 5 seconds, and the Doppler signal is weaker than yesterday.

(5) T 99.4 F, HR 84, BP 148/82, SpO2 96% on room air.

(6) Morning creatinine 2.31 (was 1.98 yesterday) and vancomycin is due at 0900.

(7) Left heel has a small area of blanchable redness; heels are floated on pillows.

(8) He ate 75% of his breakfast and drank his protein supplement.

Item numbers that need follow-up:

7.5 Priority Actions *(2 pts)*

HYPOTHETICAL: at 1130 on the day of surgery he is still NPO and nauseated, sweaty and shaky, drowsy but arousable. The meter reads 54 mg/dL. His right forearm IV is patent. Number the actions in the order you would do them.

Number the actions 1 (first) to 5 (last).

[___] Stay with him, check his airway and level of consciousness, and call for help.

[___] Give dextrose 50% 25 mL (12.5 g) IV per the hypoglycemia protocol.

[___] Recheck the glucose in 15 minutes and repeat the dextrose if it is still below 70.

[___] Notify the provider with SBAR and ask about changing the insulin doses.

[___] Document, then give a carbohydrate snack when awake and nausea is controlled and recheck glucose again within the hour.